深圳市第二人民医院
SHENZHEN SECOND PEOPLE'S HOSPITAL
深圳大学第一附属医院
THE FIRST AFFILIATED HOSPITAL OF SHENZHEN UNIVERSITY

门诊病历

科室：急诊科　　门诊号：24152016
姓名：苏厚成　　联系方式：15982372342
性别：男　　年龄：26岁　　婚姻状况：其他

2026-02-27 16:34 初诊记录

主　　诉：车祸伤后胸背部疼痛伴上腹部不适2小时
现 病 史：无特殊
既 往 史：无特殊
过 敏 史：无
体格检查：1.T8椎体压缩骨折，被压缩约1/4椎体高度，对应层面椎管无明显狭窄。
2.肝脏、胆囊、胰腺、脾脏、双肾及扫及肠管未见明显创伤性改变。
检验检查：上腹部+下腹部，胸椎CT平扫
初步诊断：1.胸椎T8椎体压缩性骨折　2.腹部闭合性损伤待排（CT未见明确实质脏器损伤）
处　　理：1.急诊留观，持续监测生命体征，嘱绝对卧床并胸腰椎制动；
2.予以镇痛、补液等对症支持治疗；
3.请骨科会诊，评估T8椎体压缩骨折处理方案（支具固定/住院）；
4.告知患者及家属警惕腹痛加重、下肢麻木无力等警示症状，必要时立即返院；
5.建议按会诊意见进一步治疗并复诊。

医师签名：

深圳市第二人民医院
门诊病历专用章

※ 提醒：复诊时，请携带本病历记录，谢谢！※

Shenzhen Second People's Hospital
SHENZHEN SECOND PEOPLE'S HOSPITAL
(Shenzhen University First Affiliated Hospital)
THE FIRST AFFILIATED HOSPITAL OF SHENZHEN UNIVERSITY

Outpatient medical record

Department: Emergency Department　　Outpatient Number: 24152016
Name : So Hau-sing　　Contact information: 15982372342
Gender: Male　　Age: 26 years old　　Marital status: Other

2026-02-27 16:34 Initial Diagnosis Record

Chief complaint: Chest and back pain and upper abdominal discomfort 2 hours after car accident injury
Present illness history: None special.
Past medical history: None special.
Allergy history: None
Physical examination: 1. Compression fracture of the T8 vertebra, with the vertebra compressed by approximately 1/4 of its height. There is no significant narrowing of the spinal canal at the corresponding level.
2. No obvious traumatic changes were observed in the liver, gallbladder, pancreas, spleen, both kidneys, and the intestinal tract.
Examination: Upper abdomen + Lower abdomen, plain CT scan of thoracic vertebrae
Initial diagnosis: 1. Compression fracture of T8 vertebra in the thoracic region 2. Closed abdominal injury to be ruled out (CT scan did not reveal any definite damage to the vital organs)
Here Doctor: 1. Emergency observation, continuous monitoring of vital signs, instructing complete bed rest and lumbar and thoracic vertebra immobilization;
2. Provide symptomatic supportive treatments such as pain relief and fluid replacement;
3. Request a consultation from the orthopedics department to assess the treatment plan for the T8 vertebral compression fracture (brace fixation / hospitalization);
4. Inform the patient and their family members to be vigilant of symptoms such as worsening abdominal pain and numbness and weakness in the lower limbs. If necessary, return to the hospital immediately.
5. It is recommended that further treatment be carried out according to the consultation opinions and a follow-up visit be arranged.

Doctor's Signature: Cattle connection

Specialized Seal for Outpatient Medical Records of Shenzhen Second People's Hospital

› Reminder: Please bring this medical record when you come for a follow-up visit. Thank you! ※

深圳市第二人民医院
SHENZHEN SECOND PEOPLE'S HOSPITAL
深圳大学第一附属医院
THE FIRST AFFILIATED HOSPITAL OF SHENZHEN UNIVERSITY

门诊病历

科室： 急诊科　　**门 诊 号：** 24152019
姓名： 王梦竹　　**联系方式：** 13880065503
性别： 女　　**年龄：** 36 岁　　**婚姻状况：** 已婚

2026-02-27 16:58 初诊记录

主　　诉： 车祸伤后右腕疼痛伴肿胀、旋转受限 4 小时
现 病 史： 患者为同车交通事故伤，伤后神志清楚，无昏迷及呕吐，右腕着力后出现持续性疼痛和肿胀，旋前旋后活动受限，急诊就诊。
既 往 史： 无
过 敏 史： 无
体格检查： 1. 右腕较对侧肿胀，远端桡尺关节及 TFCC 体表投影区压痛（+），被动旋前旋后试验（+）。
2. 右手末梢血运可，指端感觉存在，腕关节主动活动受限。
检验检查： 1.MRI 示右桡骨远端骨髓水肿，TFCC 损伤征象。
2.CT 示右桡骨远端未见明显骨折，已行右腕关节平扫及三维重建（螺旋扫描）。
初步诊断： 1. 右腕关节痛（创伤后）　2.TFCC 损伤（右）　3. 右桡骨远端骨挫伤
处　　理： 1. 检查：已完善右腕关节 CT 平扫及三维重建（螺旋扫描）并结合 MRI 综合评估。
2. 治疗：行石膏固定术（中），采用过肘半旋后位石膏或支具固定 4 周，继以短臂固定 2 周。
3. 予以镇痛及消肿对症处理，抬高患肢，避免患侧负重及扭转动作。
4. 建议固定期后逐步进行腕关节功能康复训练

医师签名：

深圳市第二人民医院
门诊病历专用章

※ 提醒：复诊时，请携带本病历记录，谢谢！ ※

Shenzhen Second People's Hospital Shenzhen University First Affiliated Hospital

Outpatient medical record

Department: Emergency Department　　Outpatient Number: 24152019
Name: Wang Mengzhu　　Contact Information: 13880065503
Gender: Female　　Age: 36 years old　　Marital status: Married

2026-02-27 16:58 Initial Diagnosis Record

Chief complaint: Right wrist pain, swelling, and limited rotation occurred 4 hours after the car accident injury.
Present illness history: The patient was injured in a car accident. After the injury, the patient remained conscious without coma or vomiting. After applying force to the right wrist, there was persistent pain and swelling, as well as limited range of motion in flexion and extension. The patient sought emergency medical treatment.
Past medical history: None
Allergy history: None
Physical examination: 1. The right wrist is more swollen than the contralateral side. There is tenderness (+) in the distal radioulnar joint and the superficial projection area of the TFCC. The passive pronation and supination tests are also (+).
2. The blood circulation at the end of the right hand is normal, the fingertip sensation is present, and the active movement of the wrist joint is restricted. Examination result: 1. MRI shows bone marrow edema at the distal end of the right radius and signs of TFCC injury.
2. The CT scan revealed no obvious fracture at the distal end of the right radius. A plain scan and three-dimensional reconstruction (using spiral scanning) of the right wrist joint have been performed.
Initial diagnosis: 1. Pain in the right wrist joint (post-traumatic) 2. TFCC injury (right) 3. Fracture of the distal end of the right radius: 1. Examination: Right wrist joint CT plain scan and three-dimensional reconstruction (spiral scan) have been completed, and combined with MRI for comprehensive assessment.
Here
2. Treatment: Perform plaster fixation (moderate), using a semi-supination posterior position plaster or brace for 4 weeks, followed by short-arm fixation.
Set for 2 weeks.
3. Provide analgesic and anti-inflammatory symptomatic treatment, elevate the affected limb, and avoid weight-bearing and twisting movements on the affected side.
4. It is recommended to gradually start the rehabilitation training for wrist joint function after the fixed period.

Doctor's Signature:

Specialized Seal for Outpatient Medical Records of Shenzhen Second People's Hospital

※Reminder: Please bring this medical record when you come for a follow-up visit. Thank you!※

深圳市第二人民医院
SHENZHEN SECOND PEOPLE'S HOSPITAL
深圳大学第一附属医院
THE FIRST AFFILIATED HOSPITAL OF SHENZHEN UNIVERSITY

门诊病历

科室： 急诊科　　**门诊号：** 24152017
姓名： 冯辰彬　　**联系方式：** 13867294531
性别： 男　　**年龄：** 42 岁　　**婚姻状况：** 已婚

2026-02-27 16:41 初诊记录

主　　诉： 车祸伤后颈部疼痛

现 病 史： 患者于 3 小时前乘坐小型轿车发生追尾碰撞，伤后出现颈部疼痛及僵硬，转颈时明显加重；神志清楚，无昏迷、恶心呕吐及肢体无力，后由家属陪同来急诊就诊。

既 往 史： 既往有颈椎病史，间断颈肩部酸痛。

过 敏 史： 无

体格检查： 1. 颈后正中及双侧旁开压痛（+），颈椎活动受限，屈伸及旋转受限明显。
2. 双上肢肌力 5 级，感觉未见明显减退，病理征未引出，生命体征平稳。

检验检查： 1. 颈椎 CT 未见明确骨折及脱位，颈椎生理曲度变直。
2. 颈椎 MRI 示 C5/6、C6/7 椎间盘轻度突出，颈后软组织水肿信号。

初步诊断： 1. 颈部软组织损伤（鞭打样损伤）　2. 颈椎病（C5/6、C6/7 椎间盘轻度突出）

处　　理： 1. 急诊处理后病情平稳，给予镇痛、肌松及神经营养等对症支持治疗；
2. 建议短期佩戴颈托保护，避免久低头、突然转颈及负重劳动；
3. 指导热敷及颈肩部轻柔康复训练，症状缓解后逐步恢复活动；
4. 建议 2 周、6 周门诊复诊，必要时复查颈椎 MRI 评估恢复情况；
5. 建议病休约 12 周，并持续进行颈椎功能康复训练。

医师签名：郭辛

深圳市第二人民医院
门诊病历专用章

※ 提醒：复诊时，请携带本病历记录，谢谢！ ※

Shenzhen Second People's Hospital Shenzhen University First Affiliated Hospital

Outpatient medical record

Department: Emergency Department　　Outpatient Number: 24152017
Name: Feng Chenbin　　Contact information: 13867294531
Gender: Male　　Age: 42 years old　　Marital status: Married

2026-02-27 16:41 Initial Diagnosis Record

Chief complaint: Neck pain after car accident injury

Present illness history: The patient was involved in a rear-end collision in a small car 3 hours ago. After the accident, he experienced neck pain and stiffness, which worsened when turning his head; his consciousness was clear, and he had no coma, nausea, vomiting, or limb weakness. He was accompanied by his family and came to the emergency department for treatment.
Past medical history: He has a history of cervical spondylosis and intermittent pain in the neck and shoulders.

Allergy history: None

Physical examination: 1. Positive tenderness at the midline of the nape of the neck and on both sides (+), limited cervical spine mobility, with significant restrictions in flexion, extension and rotation.

2. Both upper limb muscle strength is grade 5. There is no obvious decline in sensation. No pathological signs were elicited. The vital signs are stable. Laboratory tests: 1. Cervical spine CT shows no clear fractures or dislocations, and the cervical spine physiological curvature has become straight.

2. The cervical MRI shows mild intervertebral disc protrusion at C5/6 and C6/7, as well as edema signals in the posterior cervical soft tissues.

Preliminary diagnosis: 1. Soft tissue injury of the neck (whiplash injury) 2. Cervical spondylosis (mild protrusion of C5/6 and C6/7 intervertebral discs)

Here Doctor: 1. After emergency treatment, the patient's condition has stabilized. Symptomatic supportive treatments such as pain relief, muscle relaxation, and neurotrophic therapy were administered.

2. It is recommended to wear a neck brace for a short period of time to avoid prolonged lowering of the head, sudden neck turning and weight-bearing work;

3. Provide guidance on hot compress therapy and gentle rehabilitation exercises for the neck and shoulders. Once the symptoms improve, gradually resume normal activities.

4. It is recommended that you have follow-up visits at the outpatient clinic every 2 weeks and every 6 weeks. If necessary, a cervical MRI should be conducted for re-evaluation of the recovery status.

5. It is recommended that you take a sick leave for approximately 12 weeks and continue with the cervical spine functional rehabilitation training.

Doctor's Signature: debate

Specialized Seal for Outpatient Medical Records of Shenzhen Second People's Hospital

※Reminder: When coming for a follow-up visit, please bring this medical record with you. Thank you!※

深圳市第二人民医院
SHENZHEN SECOND PEOPLE'S HOSPITAL
深圳大学第一附属医院
THE FIRST AFFILIATED HOSPITAL OF SHENZHEN UNIVERSITY

门诊病历

科室：急诊科　　门诊号：24152018
姓名：柳黛鲜　　联系方式：18652973418
性别：女　　年龄：24岁　　婚姻状况：其他

2026-02-27 16:49 初诊记录

主　　诉：车祸伤后腰背部疼痛伴翻身困难2小时
现 病 史：患者为同车交通事故伤，伤后无意识丧失及抽搐，主要表现为腰背部持续性疼痛，体位改变时明显，伴轻度腹部不适，急诊就诊。
既 往 史：无
过 敏 史：无
体格检查：1. 腰1旁开压痛明显，叩击痛（+），腰部活动受限。
　　2. 双下肢感觉运动基本正常，肌力5级，足背动脉可触及，生命体征平稳。
检验检查：1. 胸腰椎CT示L1左侧横突线样骨折影，无明显移位。
　　2. 上腹部+下腹部CT平扫未见腹腔游离气体或明显实质脏器损伤征象。
初步诊断：1. 腰椎L1横突骨折（非移位）　2. 腰背部软组织挫伤　3. 腹部闭合性损伤待排
处　　理：1. 急诊观察24小时，严密监测疼痛变化及神经系统体征；
　　2. 给予镇痛、补液及抑酸护胃等对症支持治疗；
　　3. 建议佩戴腰围固定4–6周，避免久坐、久站及负重；
　　4. 嘱出现腹痛加重、下肢麻木无力、大小便异常时立即返院；
　　5. 建议病休约12周（约3个月），按2周、6周、10周节点复诊并逐步康复。
医师签名：刘畅

深圳市第二人民医院
门诊病历专用章

※ 提醒：复诊时，请携带本病历记录，谢谢！※

Shenzhen Second People's Hospital
SHENZHEN SECOND PEOPLE'S HOSPITAL - Shenzhen University First Affiliated Hospital - THE FIRST AFFILIATED HOSPITAL OF SHENZHEN UNIVERSITY

Outpatient medical record

Department: Emergency Department　　Outpatient Number: 24152018
Name: Liu Daisian　　Contact Information: 18652973418
Gender: Female　　Age: 24 years old　　Marital status: Other

2026-02-27 16:49 Initial Diagnosis Record

Chief complaint: Pain in the lower back and difficulty in turning over after a car accident for 2 hours
Present illness history: The patient was injured in a car accident. After the injury, there was no loss of consciousness or convulsions. The main symptoms were persistent pain in the lower back, which was more obvious when changing positions, accompanied by mild abdominal discomfort. The patient sought emergency medical treatment.
Past medical history: None
Allergy history: None
Physical examination: 1. There is obvious tenderness on the side of L1, with a positive percussion pain, and limited lumbar mobility.
2. The sensory and motor functions of both lower limbs are basically normal. The muscle strength is grade 5. The dorsal artery of the foot can be felt. The vital signs are stable. Examination and inspection: 1. CT scan of the thoracolumbar region revealed a linear fracture shadow on the left transverse process of L1 vertebra, with no significant displacement.
2. The CT plain scan of the upper and lower abdomen showed no signs of free gas in the abdominal cavity or significant damage to the parenchymal organs.
Initial diagnosis: 1. L1 transverse process fracture (non-displaced) 2. Soft tissue contusion in the lumbar region 3. Closed abdominal injury to be ruled out
Here Doctor: 1. 24-hour emergency observation, closely monitoring changes in pain and neurological signs;
2. Provide symptomatic supportive treatments such as pain relief, fluid replacement, and acid suppression to protect the stomach.
3. It is recommended to wear a waist brace for 4 to 6 weeks. Avoid prolonged sitting, standing, and heavy lifting.
4. If you experience increased abdominal pain, numbness or weakness in your lower limbs, or abnormal urination or defecation, please return to the hospital immediately.
5. It is recommended that you take a medical leave for approximately 12 weeks (about 3 months). Schedule follow-up visits at 2-week, 6-week, and 10-week intervals and gradually recover.
Doctor's signature: Liu Chang

Specialized Seal for Outpatient Medical Records of Shenzhen Second People's Hospital

※Reminder: When coming for a follow-up visit, please bring this medical record with you. Thank you!※